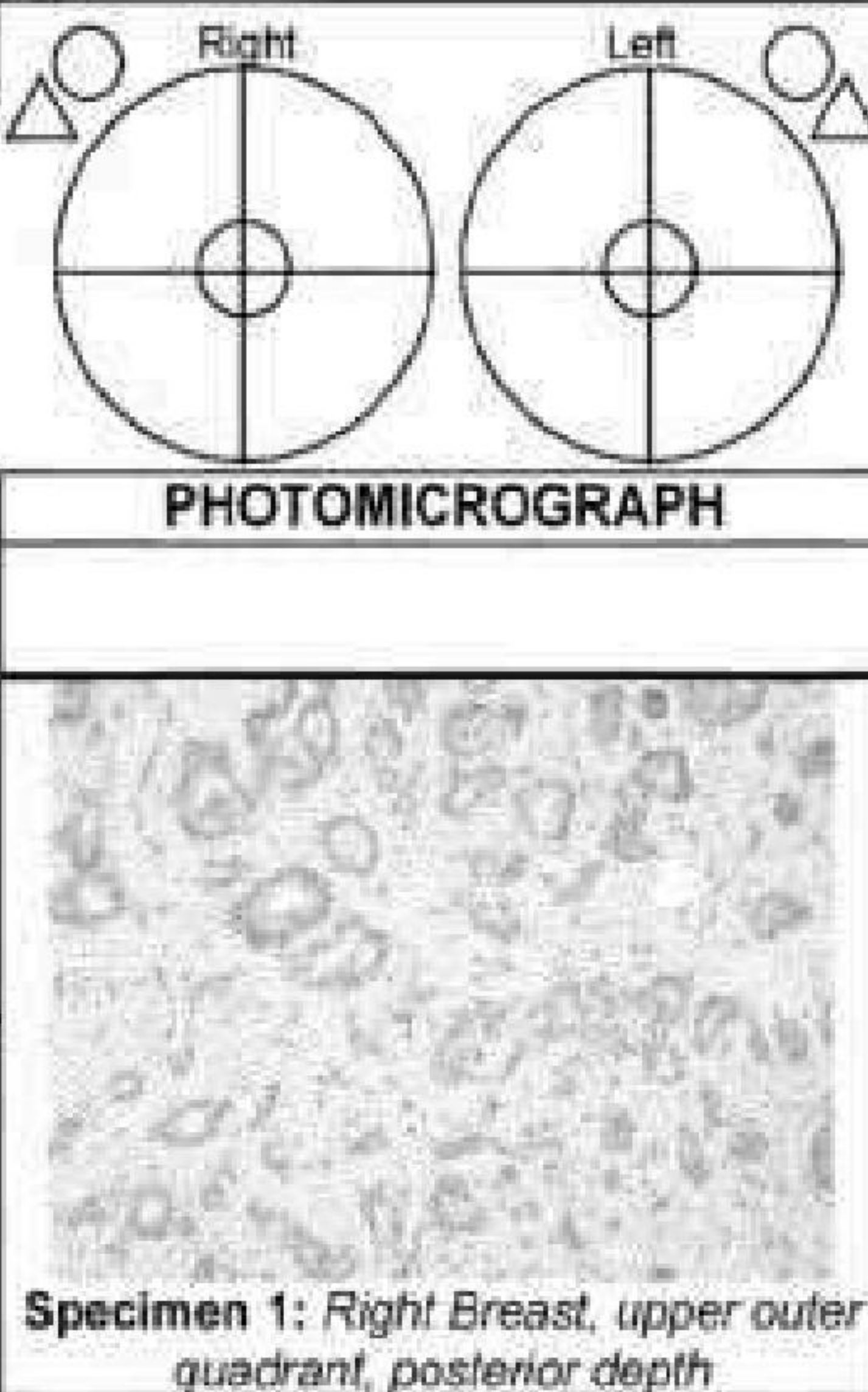


Media Information

Sonic Healthcare 9:03:20 AM PAGE 3/004 Fax Server

Wirth, Susan		DOB:		#:	
Gender: F	Age: 63	Atlantic Medical Imaging - Somers Point			
Social:					
Patient:					
Date of Collection:		Somers Point, NJ			
Date Received:		(609) 653-0209			
Clinical Information: 63 year old Female with enhancing 7 mm mass also seen to enhance on MRI Right Breast, upper outer quadrant, posterior depth; stop light clip marker; Mammotome; Stereotactic					

ADDENDUM Breast Pathology Report	
RESULTS	
1: Diagnosis <i>Right Breast, upper outer quadrant, posterior depth: Moderately differentiated invasive ductal carcinoma (tubule formation 3/3, nuclear pleomorphism 2/3, mitotic count 1/3), measuring 0.6 cm in maximal length in this material.</i> Gross Description: Received in formalin labeled with the patient's name are multiple light tan to yellow tan soft tissue fragments measuring 2.5 x 2.5 x 0.3 cm in aggregate. The entire specimen is submitted in 3 cassette(s) labeled "1A-1C". Ischemic time: 0 hrs: 25 min; fixation time: 19 hrs: 5 min. yellow/po	RightLeft  PHOTOMICROGRAPH Specimen 1: Right Breast, upper outer quadrant, posterior depth

Comments
Prognostic/predictive marker studies are being performed; the results will follow in an addendum. Following CAP/ASCO guidelines HER2 FISH will be performed on all invasive breast carcinomas if HER2 IHC is 2+ (EQUIVOCAL). Case reviewed in departmental conference.

A copy of this report has been sent to the referring physician, been supplied by the requesting physician's office.

DOB:

#

10:00:52 AM

Studies for calponin and p63 underscore the absence of myoepithelial cells (MEC) in the area of invasive cancer.

Loss of membranous E-cadherin and B-catenin immunostaining supports the additional finding of atypical lobular hyperplasia (ALH) and lobular carcinoma in situ (LCIS), nuclear grade 2, classic type. Atypical ductal hyperplasia (ADH) is also present.

The invasive carcinoma demonstrates:

Estrogen receptor (ER): POSITIVE - >90% strong intensity nuclear staining:

Progesterone receptor (PgR): POSITIVE - >90% strong intensity nuclear staining;

HER-2 neu score: 1+/-low (NEGATIVE) (faint/incomplete membranous staining observed in >10% of tumor cells)*.

Proliferation index (Ki-67): 15% nuclear staining.

*Patients with breast cancers that are HER2 IHC 1+ or IHC 2+/ISH not-amplified may be eligible for a treatment that targets non-amplified/non-overexpressed levels of HER2 expression for cytotoxic drug delivery (IHC 0 results do not result in eligibility currently).

Wolff AC, Somerfield MR, Dowsett M, Hammond MEH, Hayes DF, McShane LM, Saphner TJ, Spears PA, Allison KH. Human Epidermal Growth Factor Receptor 2 Testing in Breast Cancer. Arch Pathol Lab Med. 2023 Sep 1;147(9):993-1000. doi: 10.5858/arpa.2023-0950-SA. PMID: 37303228.

ER clone (SP1) ultraview DAB detection; computer assisted quantitative IHC using Leica Aperio AT2 and Leica ImageScope

PgR clone (1E2) ultraview DAB detection; computer assisted quantitative IHC using Leica Aperio AT2 and Leica ImageScope

Ventana Pathway HER2 neu/4B5 ultraview DAB detection

K167 clone MIB-1 (Dako); computer assisted quantitative IHC using Leica Aperio AT2 and Leica ImageScope

Image analysis is dependent on the handling and processing of the tissue, quality of staining, density of tumor cells, the presence of stromal infiltrate, and staining artifacts. The clinical interpretation of any positive staining, or its absence, must be evaluated within the context of clinical history, morphology, and other histopathological criteria.

Negative and positive (internal if applicable) controls show appropriate results

This evaluation is performed according to guidelines issued by CAP/ASCO.

Addendum Sign-Out

Addendum Issued By: Claudine Habib M.D.

Electronically signed out by:

Dr. Claudine Habib

(877) 258-9310

Report Date:

Images included in this report are for information only and are not intended for diagnosis.

Some tests performed at CBLPath Inc. have not been cleared or approved for specific uses by the U.S. Food and Drug Administration (FDA). The FDA has determined that such clearance or approval is not required, this laboratory has verified the validity and accuracy of these tests for clinical purposes. CBLPath is registered under the Clinical Improvement Amendments Act of 1998 (CIAA) as qualified is

Direct and Indirect

Specimens processed and interpreted at CBLPath, Inc., 760 Westchester Ave,
Rye Brook, NY [REDACTED] (877) 225-7284. Dr. Ciri-Christina Rizk,
Laboratory Director, NY PFI# 3954; CLIA# [REDACTED] CAP LAP# 7184143.

Wirth, Susan

Page 2 of 2

Accession #:

A copy of this report has been sent to the referring physician, [REDACTED] [REDACTED] by the requesting physician's office.

Other: Outside Pathology

ADDENDUM BREAST PATH

00:00

Attached To:

OUTSIDE PATHOLOGY ORDER

Orders Only on with Historical Provider,

Source Information

Priscilla P. Hazel | Msk Pas